



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نرمين عبد الله ابو السعود عباس

المحافظة : القليوبية الحي / المدينة : شبرا الخيمة ثان العنوان : *

رقم البطاقة : 29209071400625 رقم التليفون : 01141444216 تاريخ تسجيل الإستمارة : 04-10-2026

العمر (تاريخ الميلاد) : 07-09-1992 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي التوقيع: (-----) المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع: (-----)
الاسم: د/ندى حسن الوظيفة: التوقيع: (-----)
الاسم: د/نرمين رمضان الوظيفة: التوقيع: (-----)

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نزمين عبد الله ابو السعود عباس National ID: 29209071400625	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
تدقيق



DMS
تدقيق

MDT FORM

Name: لرمين عبدالله ابو السعود حسان	MRN: 9752
Age: 34 Y	ID: 29209071400625
Menopausal Sta: PRE	Phone: 01114444216
NRS SCORE: PS0	Marital Status: MARRIED
BMI:	Family History:
CO Morbidity: NO	Reason Of MDT: NACT

Clinical Data:

BX: **IDC GIII**

ER: **0/8** **PR:** **0/8** **HER2:** **0** **KI67:** **35%**

Sonomammography:

Mets Workup:

CT CAP: demonstrated a right breast soft tissue mass measuring 3.3 cm with a right axillary lymph node measuring 1.7 cm, with no evidence of distant metastasis; an incidental left adnexal cyst measuring approximately 5 x 3 cm was noted, likely benign.

BONE SCAN :FGREE

History:

clinical stage is cT2N1? M0. >> US REQUESTED FOR CONFIRMATION OF NODAL

Recommendations:

MDT Outcome & Plan

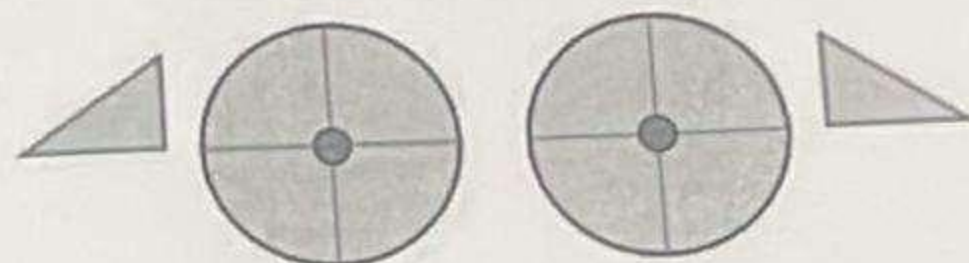
BRCA TEST , CLIP INSERTION , PORTACATH >>> PENDING

for NACT with Pembrolizumab X4+TAXOL ,CARBO X 12 WEEKS

followed by Pembrolizumab X 4 + AC for 4 cycles,

+ ZOLADEX -WLE-SNLD& FOR CLINICAL ASSESSMENT AT SURGERY CLINIC BEFORE STARTING NACT TO ASSESS NEED OF CLIP INSERTION

R



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد كوالاني	د/ املية عرسوي	د/ لرمين رمضان			

Name:	نرمين عبدالله ابو السعود عباس	
MR. :	9752	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	
	رقم طبي:	
بقسم:		
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	
تاريخ وساعة الدخول :		
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presents with a history of right breast cancer, stage II (T2 N1 M0), triple-negative, Ki-67 35%, IDC grade 3. At initial evaluation (23/04/2026) the patient had a 4 × 4 cm right upper-quadrant mass and a 1 × 1 cm mobile axillary node. Clips were inserted during the exam. The planned neoadjuvant regimen was weekly taxol 160 mg + carboplatin AUC 2, with keytruda approval pending and zoladex as endocrine therapy. On 02/05/2026 keytruda approval remained pending; the patient continued weekly taxol 160 mg + carboplatin AUC 2 for weeks 2 and 3. On 09/05/2026 the patient received week 3 taxol 160 mg + carboplatin AUC 2. Weight remained 92 kg, height 156 cm, BMI 1.99, abdominal weight 66.1 kg. On 23/05/2026 the patient received week 5 taxol 160 mg + carboplatin AUC 2; keytruda approval status was still uncertain. On 31/05/2026 the patient received week 6 taxol 160 mg + carboplatin AUC 2; laboratory values were accepted on 16/5. On 06/06/2026 the patient experienced a grade 1 pruritic hypersensitivity reaction to carboplatin last week. Examination showed a 2 × 2 cm right upper-quadrant mass. The patient received week 7 taxol 160 mg + carboplatin AUC 2 over 2 h with solucortef premedication. Weeks 8 and 9 were pending keytruda approval. On 27/06/2026 the patient's right upper-quadrant mass remained 2 × 2 cm. The patient received week 9 taxol 150 mg + carboplatin and weeks 10 and 11 of paclitaxel (ph) with an 63 % response noted on 27/4. On 19/07/2026 the patient's mass remained 2 × 2 cm. The patient received week 12 taxol 150 mg + carboplatin AUC 2 as part of the neoadjuvant course. On 02/08/2026 the patient's mass remained 2 × 2 cm. The patient received cycle 1 of adriamycin 120 mg/m² + endoxan 1200 mg as part of the neoadjuvant regimen. On 16/08/2026 the patient received cycle 1 of keytruda 200 mg today; the dose was postponed by one week due to ANC 0.6. On 23/08/2026 the patient's mass measured 20 × 15 mm with a partial radiological response (63 % reduction) noted on 27/4. The patient received cycle 1 of keytruda 200 mg and cycle 2 of adriamycin 120 mg + endoxan 1200 mg. The right upper-quadrant mass measured 2 × 1 cm on exam.		
تحريراً في ,		
	الوقت :	21-09-2026
		التاريخ :
		الطبيب المعالج :
		dina makram
يعتمد ,		

The patient has completed 1 cycle of keytruda and 2 cycles of adriamycin + endoxan to date.

Patient Name : نرمن عبدالله ابوالسعود عباس
Patient No. : 29209071400625
Path No. :
Sex : Female
Age : 33 Y
Referring Doctor:

Request No. : 1836209
Request Date : 07/03/2026
Verification Date : 28/03/2026
Financial Category : معهد ناصر - وزارة الصحة
Department : مبادرة ١٠٠ مليون صحة
IHC No. : 205/26

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL \ RADIOLOGIC DATA

Right breast mass, tru - cut biopsy, paraffin block.

GROSS PATHOLOGY

Received one Paraffin block labeled (864)

IHC technique:

Sections were prepared from the paraffin blocks, processed in benchmark ultra machine (ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter - stain.

HISTOPATHOLOGY

IMMUNOHISTOCHEMISTRY RESULT:

DIAGNOSIS

Right breast mass, tru - cut biopsy, paraffin block:

- ER: Negative (0/8).
- PR: Negative (0/8).
- Her2: Negative (score 0).
- Ki67: 35%.

Signature

Dr. Amal Ahmed Hareedy.

25/03/2026

Patient Name : نرمن عبدالله ابوالسعود عباس
Patient No. : 29209071400625
Path No. : 864/26
Sex : Female
Age : 33 Y
Referring Doc :

Request No. : 1819543
Request Date : 09/02/2026
Verification Date : 19/02/2026
Financial Category : معهد ناصر - وزاره الصحة
Department : مبادرة ١٠٠ مليون صحة

Pathology Report

Clinical Data: Right breast mass.
Nature of the Specimen:
U/S guided Tru cut biopsy.

Gross Appearance:

Specimen contained three fragmented grayish tissue cores ranging: 11-15 mm long, totally submitted.

Microscopic Appearance:

Sections reveal a breast tissue infiltrated by poorly differentiated invasive ductal carcinoma composed of nests and cords of cells with focal ductal differentiation, surrounded by stromal desmoplastic reaction. Tumor cells are large, pleomorphic, with marked anaplasia and high mitotic activity. There is associated dense lymphocytic infiltrate. There is no associated significant intraductal component (DCIS).

Diagnosis:

RIGHT BREAST MASS; TRU CUT BIOPSY:
- Invasive duct carcinoma, grade III.

Prof.Dr/ Osama Mostafa

Patient Name: Nermeen Abdelah aboAl
Soad

Date: 15/3/2026

^{99m}Tc -MDP BONE SCAN

Referring physician: Prof.

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Heterogenous abnormal radiotracer uptake is seen at the spine , likely of spondylodegenerative changes) ,
- Other wise , No scientigraphic evidence of bony metastases .
 - Osteoarthritic changes are seen at, both shoulders , both knees .
- Both kidneys are normally seen .

Conclusion:

No defenite active osseous metastatic lesions could be seen all over the surveyed skeleton , .

Nuclear Medicine Unit

DR. Sherin Mahdy, MEd

DR/ Mohamed Emam

أ.م.د. محمد إمام
٢٠٢٦ / ٣ / ١٥



Patient Name: نرمن عبدالله أبو السعود عباس	Patient ID: 451780
Study Date: 17-Mar-26 9:40:00 AM	

CT chest, abdomen & pelvis with contrast:-

Chest:

- Right breast outer aspect 3.3cm heterogeneously enhancing soft tissue mass lesion (proved invasive ductal carcinoma).
- Right axillary LN about 1.7 cm.
- Clear both lungs with no definite focal lesions, infiltrations or bronchiectatic changes.
- Clear both costophrenic recesses.
- Normal cardiac size with no pericardial effusion.

Abdomen & pelvis:

- Mild enlarged left hepatic lobe with no definite hepatic focal lesions or dilated IHBR. Patent main portal vein and its central branches.
- IUCD is seen in place.
- Supra pubic anterior abdominal wall midline thick scar tissue.
- Left sided pelvic cyst 5X3 cm likely adnexal.
- Normal CT appearance of the spleen, adrenal glands, pancreas, kidneys, IVC and aorta.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Normal other pelvic organs.

Typist

PROF: DR MOHAMED ABD ELLATEF.

HEND

Spelling checked by



Name: nermein Abdullah aboalsaoud

Date: 2/2/2026

Bilateral 3D Full Field Digital Mammography

Clinical Summary :

Technique:

Mediolateral and Craniocaudal views
High resolution US.

Findings:

RIGHT BREAST:

Scattered glandular pattern (ACR B)
UOQ irregular dense lesion by US it corresponding to non-circumscribed ,
irregular hypoechoic lesion , seen at 11 Oclcok , 7 cm from the nipple and
measures 35X27 mm
Indeterminate axillary L.Ns with thickened cortex

LEFT BREAST:

Scattered glandular pattern (ACR B)
No masses, assymetries, distortion or suspicious microcalcifications.
US demonstrated normal glandular and fatty tissue
No solid or cystic masses.
Normal/skin/nipple/areolar complex.
Non-specific axillary L.Ns , with normal texture and preserved hilum

IMPRESSION:

Right side : Malignant looking lesion with suspicious pathological LNS (BIRAD V)

Left side: normal study (BIRAD I)

Best Regards,
Dr Manal Ibraheim, MD

ECHO REPORT

NAME: محمد عبد الله أبو السعود

DATE: 27/4/2026

ID NUMBER: /

SEX: ♀ AGE: 33

EF	63.1	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.8	3-3.7cm
LVESD		2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *normal*

LV Wall Thickness *normal*

LV Diastolic Function *normal*

SWMA *0.85 A*

LA and Ao Root Dimensions *normal*

Mitral Valve *normal*

Aortic Valve *normal*

Tricuspid Valve *normal TR*

Pulmonary Valve *normal*

Right Side and Systolic Function *normal*

Intracardiac Masses and Effusion *normal*

DR

[Signature]